STATE OF TENNESSEE
DEPARTMENT OF HEALTH

IN THE MATTER OF:	)	**BEFORE THE TENNESSEE BOARD OF MEDICAL EXAMINERS**
ERIC WELLING, M.D.	)	
RESPONDENT	)	**CASE NO. 17.18-231717A**
SALT LAKE CITY, UTAH	)	
TENNESSEE LICENSE NO. 65466	)	

AGREED ORDER

Come now the Division of Health-Related Boards of the Tennessee Department of Health ("State"), by and through the Office of General Counsel, and Respondent, Scott Sang-Hyun Lee, M.D., ("Respondent"), and respectfully move the Tennessee Board of Medical Examiners ("Board") for approval of this Agreed Order effecting Respondent's medical license in the State of Tennessee.

The Board is responsible for the regulation and supervision of medical professionals licensed to practice in the State of Tennessee. *See* Tennessee Medical Practice Act, Tennessee Code Annotated sections ("Tenn. Code Ann. §§") 63-6-101, *et seq*. It is the policy of the Board to require strict compliance with the laws of this State, and to apply the laws so as to preserve the quality of medical care provided in Tennessee. It is the duty and responsibility of the Board to enforce the Tennessee Medical Practice Act in such a manner as to promote and protect the public health, safety and welfare in every practicable way, including disciplining individuals who violate the provisions of Tenn. Code Ann. § 63-6-101, *et seq*. or the Rules and Regulations promulgated by the Board and recorded in the *Official Compilation Rules and Regulations of the State of Tennessee* ("Tenn. Comp. R. & Regs.")

Respondent, by his signature to this Agreed Order, waives the right to a contested case hearing and any and all rights to judicial review in this matter. Respondent agrees that

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presentation to and consideration of this Agreed Order by the Board for ratification and all matters divulged during that process shall not constitute unfair disclosure such that the Board or any of its members shall be prejudiced to the extent that requires their disqualification from hearing this matter should this Order not be ratified. Likewise, all matters, admissions and statements disclosed or exchanged during the attempted ratification process shall not be used against Respondent in any subsequent proceeding unless independently entered into evidence or introduced as admissions. Although Respondent disputes that the stipulations of fact support grounds for discipline, he enters into this Agreed Order for the purposes of avoiding time and expense of further administrative proceedings and litigation.

I. STIPULATIONS OF FACT

1. Respondent has been at all times pertinent hereto licensed by the Board as a medical doctor in the State of Tennessee, having been granted license number 65466 by the Board on February 28, 2022, through the Interstate Medical Licensure Compact ("IMLC").

2. On November 4, 2022, the Wyoming Board of Medicine summarily suspended the Respondent's medical license. As a result, the Tennessee Board of medical examiners suspended Respondent's medical license on January 3, 2023.

3. On December 14, 2023, the Wyoming Board of Medicine took final disciplinary action against Respondent's medical license with the following findings:

A. Documentary Evidence

Suspension of Dr. Welling's Clinical Privileges at Davis Hospital

I. In 2021, the Davis Hospital and Medical Center (Davis Hospital) in Layton, Utah, employed Dr. Welling as an anesthesiologist.

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II. In 2021, Dr. Welling demonstrated a higher-than-average discrepancy rate related to his use of medications, including controlled substances in his anesthesia practice.

III. On March 9, 2021, Dr. Welling was placed on a Focused Professional Practice Evaluation (FPPE).

IV. On May 25, 2021, Dr. Welling was placed on Conditional Staff Category due to continued concerns with his drug administration, medication stewardship and medical documentation.

V. On September 21, 2021, at the end of his six (6) month FPPE, Dr. Welling disclosed to the Medical Executive Committee (MCE) that he may have underlying deficits which could be the cause of his substandard performance.

VI. Dr. Welling failed his FPPE even after multiple interventions were put in place to help him improve his charting methods.

VII. In September 2021, Dr. Welling continued to have discrepancies between documentation in the electronic medical record (EMR) and what was pulled from the cart resulting in fraudulent billing and documentation.

VIII. On September 23, 2021, the Davis Hospital suspended Dr. Welling's clinical privileges due to patient health and safety concerns and was requested to obtain a cognitive assessment.

IX. After his suspension went into effect, additional patient safety concerns were raised, and Davis Hospital initiated an investigation into his performance.

X. Dr. Welling did not complete all requirements of his suspension.

Dr. Welling Presents to Dr. Van Leeuwen- Psychiatrist

XI. Benjamin N. Van Leeuwen, M.D., is a Board-certified Psychiatrist.

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XII. On September 14, 2021, the day that Davis Hospital suspended his clinical privileges, Dr. Welling presented to Dr. Van Leeuwen.

XIII. On his intake paperwork when asked about being diagnosed with a mental health condition by a medical provider, Dr. Welling handwrote "No diagnosis-Self treated." He disclosed other self-prescribing medications.

XIV. Dr. Welling reported to Dr. Leeuwen that:

> [H]e is here today for help with some issues with inattention. Has really noticed it over the past 3-4 years. Says that he has been really distracted at work. Struggles with being able to pay attention and get focused at work. He says that he has been told that he has a reputation of not paying attention to his patients. He says thar his biggest issue is that he really gets distracted by hid phone. Gets all of his work done and then immediately starts looking at his phone.
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> Understands that it represents a significant legal liability for the hospital and for himself. He feels that he does have more issues on longer cases. Has significant difficulty focusing. At his worst, states that his error rate was 30-45%. Compared to the normal error rate of 0-15%.
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> As a result of the failed review, states that the MEC has suspended him, with several stipulations.
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> Also, appears that he really has struggled with attention for much of his life. Has been able to compensate, but then changes likely due to age and other changes in work environment (new EMR, ect) have made previously effective compensatory strategies less successful. For now, will make some medication adjustments to target anxiety. Will also consider directed ADHD tx at next visit. Holding off for now pending work mandated evaluation.

XV. Dr. Welling presented to Dr. Van Leeuwen on October 18, 2021; November 16, 2021; February 15, 2022; May 9, 2022; December 2, 2022; and May 4, 2023.

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XVI. Dr. Van Leeuwen noted that Dr. Welling has been diagnosed with cyclothymia (ICD-10 code F34.0) and generalized anxiety disorder (ICD-10 code F41.1).

XVII. On December 7, 2021, after only two (2) appointments, Dr. Van Leeuwen signed a letter for the Utah State Department of Commerce DOPL (Utah Board), which stated that, "[i]t is my opinion that Dr. Welling is currently fit to practice medicine without restrictions.

XVIII. In a letter dated May 9 and December 2, 2022, Dr. Van Leeuwen noted that:

> Please note that I am not a psychotherapeutic provider, and can therefore not comment on whether or not he has engaged with appropriate talk therapy as recommended in the full neuropsychological evaluation completed by Acumen Assessments. Regarding his current ability to work, I cannot say whether he is able to perform adequately as a practicing anesthesiologist as I have not evaluated his performance in this setting, nor could I be considered an expert enough to evaluate his skills and abilities.

XIX. In a letter to the Board dated May 24, 2023, Dr. Van Leeuwen noted that:

> It is important to note that as a psychiatrist, my expertise lies primarily in assessing and managing mental health conditions. Therefore, I have no evaluated Dr. Welling's ability to function as an anesthesiologist, nor could I be expected to do so, given the specialized nature of his field.
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> If he continues to maintain his current level of stability and **adheres to the recommendations of his healthcare providers**, it is reasonable to expect that he can function at a level commensurate with his education and training as an anesthesiologist.
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> Please note that this letter serves only as a summary of Dr. Welling's mental health status and **should not be considered a comprehensive evaluation or endorsement of his professional competence in the field of anesthesiology.** In my practice as a psychiatrist, I focus on assessing and managing psychiatric conditions rather than evaluating specific professional abilities. **Should that evaluation be needed, it should be done in a clinic that specializes in forensic evaluation.**

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XX. Davis Hospital referred Dr. Welling to Acumen for a *Forensic Professional Fitness to Practice Evaluation*.

XXI. On October 6, 2021, before the assessment, Dr. Welling sent an e-mail stating that the problems with his discrepancies and inaccuracies are caused by the systems of documentation being separate and unable to "communicate with each other, "which does not burden the other anesthesiologists.

XXII. On October 11-14, 2021, Dr. Welling was evaluated at Acumen by a multidisciplinary team.

XXIII. Dr. Welling self-reported to Acumen that he has been having difficult reconciling medications on the Pyxis, anesthesia charting, and the hospital EMR, which do not talk to each other. He also disclosed that he gets distracted by his phone and then will go down the rabbit hole when he's on-call (next cases) or when looking at the news. He also stated that he has sloppy record-keeping.

XXIV. Collateral information from Davis Hospital indicates that during an audit of Dr. Welling's charts around a year ago, it was discovered that:

a. He was completing all of his post-anesthesia charts while his patients were still in surgery;

b. He completed chart entries for some patients on cases that had not even started, meaning that preanesthesia vitals and other pre-op details had already been entered;

c. This practice of pre-filling entries were on all of his patients for several weeks;

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d. His notes said that the patient was table, vitals were normal (and this data was noted to have been contrived), and the patient was recovering nicely; however, these notes were completed even though these patients were still in surgery;

e. Back to 2019, Dr. Welling was described as disorganized, leaving the room/cart a mess, defensive and insulting regarding oversight, mislabeling syringes, having persistent documentation problems, and concerns regarding infection;

f. Dr. Welling is noted as having a longstanding problem with charting which he often externalizes blame for his difficulties by blaming his phone, the computer system, hearing problems, external noise in the room, ect.; and

g. There is a trend of judgment problems, falsifying records, non-compliance with hospital polices and procedures, potential patient care problems due to inaccurate records, and an inability to self-correct.

XXV. Dr. Welling describes situations where he finds himself pinned into a corner by either external challenges that explain why is having problems, but at the same time comes across as a victim to either these things or his distractibility. Dr. Welling indicated that his problems have been phone-related, his addiction to his phone, and his belief that he has ADHD.

XXVI. Dr. Welling came into the assessment convinced that his issue stemmed from undiagnosed ADHD; however, the results of the evaluation do not support that

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diagnosis. Instead, Dr. Welling's problems stem from a previously undiagnosed mood disorder, Cyclothymia, with Anxious Distress.

XXVII. Dr. Welling self-prescribed Prozac (to deal with stress better), Lexapro 30mg qd, Wellbutrin 150 XL qd (started two years ago), Strattera (for three days). Dr. Welling self-prescribed Fluoxetine back in 1995-2000. He obtained a genetic study on his son, which suggested Bupropion may work for him, so he switched himself onto Bupropion. Later, Dr. Welling prescribed his son Lexapro. Dr Welling has been treating himself (self-prescribing) various antidepressants over the years.

XXVIII. It is clear that Dr. Welling does not understand that prescribing to a family member is considered unethical conduct.

XXIX. Dr. Welling seems oblivious to his poor judgment, misguided assumptions about his practice and persona life, and the seriousness of his misconduct-fraudulent documentation reported by David Hospital. He does not seem to appreciate his grandiose expectations or the illogic of his decisions. His insight into his problems is poor and his judgment has been poor for some time.

XXX. During the assessment and testing, Dr. Welling demonstrated a tendency to be distracted by his phone. On multiple occasions, his phone rang during testing. On one occasion, he was observed on his phone while actively engaging in a Rorschach test. When asked about his phone, he indicated that he was trying to make sure his phone was off and he did not realize he had not completed the tasks. Dr. Welling had his cell phone connected to his hearing aids. "Why

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an individual undergoing a fitness to practice evaluation would have their cell phone on during a cognitive testing is an indication of poor judgment."

XXXI. After the assessment, Dr. Welling demonstrated the degree to which he is vulnerable to distort reality in the service of hi own needs-done so in the service f positive impression management. Some of his e-mails were a bit unhinged and consistent with someone with an untreated mood disorder and Cluster B Personality Disorder-the latter of which includes lapses in reality testing, errors in reasoning, poor judgment, poor insight, the utilization of narcissistic defenses, and argumentativeness. His hypomanic style coupled with anxiety in the direction that suits him rather than sitting back and objectively assessing a given situation. He comes across as more concerned about himself than the patients who could have suffered harm due to the problems noted in the collateral information.

XXXII. On November 8, 2021, the Acumen issues its *Forensic Professional Fitness to Practice Evaluation* (Acumen Report), which provided:

Diagnoses:

Based on the results of the evaluation, the following diagnoses are offered based on DSM-V criteria:
Cyclothymia, Late-Onset, with Anxious Distress

Cluster B Personality Disorder

Conclusions:

Dr. Welling requires proper medication and psychotherapy to address the problems and challenges outlined above. Until these issues are resolved, where he acquires a stable mood and construct a reasonable plan of action to prevent future documentation and intraoperative medication management problems, we find him unfit to practice.

It is our understanding that Dr. Welling is under the care of Dr. Van Leeuwen. He is probably in the best position to decide when DR. Welling can return to

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work from a psychiatric standpoint. We imagine that he will need to produce a return to work plan approved by Davis Hospital before returning to work.

Recommendations:

Dr. Welling may benefit from a trial of Lamictal as primary mood stabilizer. It will take time to titrate him up to a therapeutic dose, so an augmentation strategy with a second-generation antipsychotic medication may be helpful. It is not clear if Wellbutrin and Buspar are doing much. Dr Whipple suggests discontinuing these medications to simplify the regimen. At this point, we would address his difficulties as if he is developing a late-life bipolar condition.

1. Dr. Welling should take a leave of absence from work until further assessment demonstrates an improvement in his thought organization on the medication.
2. Dr. Welling would benefit from an intensive partial hospital or day treatment program for professionals in crisis.

3. We recommend that Dr. Welling acquire a local psychologist to address the personality issues noted in this report.

4. We recommend that Dr. Welling sign releases of information to facility sending this report to his providers.

5. We recommend that Dr. Welling refrain from self-prescribing medications.

6. The assessment team recommends that Dr. Welling practice all time in full accordance with relevant federal, state, local, organizational, and professional regulations, ethical guidelines, and best practices. If he ever is unable or unwilling to adhere to these recommendations, we recommend that he disengage himself from the practice of medicine immediately.

Procedural History-Board Complaint

XXXIII. On October 25, 2021, the Board office received notice regarding Dr. Welling's suspension of his clinical privileges.

XXXIV. On October 27, 2021, Board staff opened complaint No. 1274 and requested a response from Dr. Welling.

XXXV. On November 12, 2021, the Board office received Dr. Welling e-mailed response:

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I am writing to verify that my privileges have been suspended at Davis Hospital pending an investigatory suspension related to anesthesia charting errors. The Davis Hospital Medical Executive Committee required an evaluation at Acumen Assessments in Lawrenceville [sic], KS for neurocognitive testing to diagnosis a medical condition that would explain anesthesia charting errors with relation to the medications removed from the pyxis drug dispensing system. I suggested that perhaps that a mental illness (ADHD) may explain my the [sic] errors and apparent distraction. The first available appointment at Acumen Assessments was 3 ½ weeks after the suspension. It was also known that Acumen Assessments require 4-6 weeks for completion of their "written review" which is what the Davis Hospital MCE required for their investigatory suspension.

I finished the assessment at Acumen Assessments on October 14. The preliminary report was a diagnosis of Cyclothymia with Anxious Distress. I am seeking consultation with Dr. Benjamin Van Leeuwen, M.D. (Psychiatrist, Layton, UT) who has changed my medication regimen.
We are still awaiting the final report from Acumen Assessments at which the Davis Hospital MEC will make their recommendations and treatment plan.

Thank you for you notification in this matter.

Dr. Welling's Medical Leave of Absence from Davis Hospital

XXXVI. On November 15, 2021, Dr. Welling requested a medical leave of absence from Davis Hospital for an indefinite period of time.

XXXVII. On November 16, 2021, the Medical Executive Committee approved his request.

XXXVIII. On November 23, 2021, the Board of Trustees finalized Dr. Welling's request for an indefinite medical leave of absence.

Procedural History-Board Requiring Mental Health Evaluation

XXXIX. On December 15, 2021, the Board office received the Acumen Report.

XL. On December 16, 2021, Rene Hinkle, M.D., and Diane Noton, M.D., were appointed as Interviewers (Interviewers) to further investigate this matter.

XLI. On or about March 2, 2022, after many requests, the Board office received medical records from Dr. Vann Leeuwen. The records indicated that Dr.

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Welling was going against Acumen Assessments recommendations by changing his own medications, not participating in an intensive partial hospital or day treatment for professionals in crisis, and not taking a leave of absence as recommended until further assessment demonstrates an improvement in his thought organization on the medication.

XLII. On March 25, 2022, the Board's Prosecuting Attorney sent a letter to Dr. Welling asking him to specifically answer how he had strictly complied with each of the seven recommendations made in the Acumen Report.

XLIII. On March 29, 2022, the Board office received Dr. Welling's e-mailed response detailing each of the Acumen recommendations and how he felt he had strictly complied with the recommendations:

I am responding to your letter of March 25, 2022. Acumen Assessments made several conclusions based upon their evaluation which you referenced. I have included these recommendations below along with my response of how I strictly complied with their recommendations and opinions.

Forensic Evaluation Summary Report

1. Dr. Welling may benefit from a trial of Lamictal as primary mood stabilizer. It will take time to titrate him up to a therapeutic dose, so an augmentation strategy with a second-generation antipsychotic medication may be helpful. It is not clear if Wellbutrin and Buspar are doing much. Dr Whipple suggests discontinuing these medications to simplify the regimen. At this point, we would address his difficulties as if he is developing a late-life bipolar condition.

What they did not include in these recommendations was their opinion that the antidepressants that I was taking prior to this assessment were, in their opinion, worsening my distraction from the Cyclothymia which they subsequently diagnosed. They suggested an immediate cessation of the antidepressants and administration of the correct treatment, a mood stabilizer. As soon as I received their verbal conclusion of Cyclothymia (October 14, 2021), I called my psychiatrist, Dr. Van Leeuwen, and mentioned to him their findings. In my next appointment with Dr. Van Leeuwen (October 19, 2021), we together reviewed the mood stabilizers and determined that many of them had serious potential side effects and life-threatening risks. He then discontinued the other antidepressants and stared me on Abilify which he was

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using as a mood stabilizer. I quickly noticed the improvement with Abilify. I was able to think more clearly and organize my thoughts better. I did not seem to have the "ups and downs" in my mood that I previously had. In addition, despite having lost my career at Davis Hospital, I seemed to handle that disappointment much better and without incapacitating depression.

2. Dr. Welling should take a leave of absence from work until further assessment demonstrates an improvement in his thought organization on the medication.

I did take a "leave of absence" and did not work in a hospital or healthcare environment for four weeks. There were several weeks in November and December where I did not work at all. I used this time to write notes about the observations that were raised by my peers in the MEC and review areas of their concern and how I could address those problems in the future. At present, I have not worked in an operating room since December 12, 2021. I work everyday in my office trying to find my next position as an Anesthesiologist. So I have complied with the recommended "leave of absence" and have tried to utilize my time efficiently and effectively to find additional employment, but also to improve my anesthesia skills with reviewing anesthesia literature and anesthesia recommendations.

3. Dr. Welling would benefit from an intensive partial hospital or day treatment program for professionals in crisis.

Acumen recommended some type of ongoing care-their specialty is to use the Assessments to self-refer patients for their day treatment programs. This is not only a recommendation but also an advertisement for their services. They spoke with Dr. Van Leeuwen on the phone several times during my assessment and felt that he was an excellent choice to monitor my progress and treatment. I have complied with all visits to Dr. Van Leeuwen. After the initial correction to my medications, I have stabilized and no other adjustments have seemed indicated. I continue to take Abilify and escitalopram as Dr. Van Leeuwen originally prescribed.

4. We recommend that Dr. Welling acquire a local psychologist to address the personality issues noted in this report.

I am continuing to see Dr. Van Leeuwen. In addition to monitoring my prescriptions, Dr. Van Leeuwen provides Cognitive Behavioral therapy in his appointments. Initially, our visits were scheduled every month. He has now scheduled a consultation every 4-6 months.

5. We recommend that Dr. Welling sign releases of information to facilitate sending this report to his providers.

I complied with this request and released all my information including the Acumen Assessment report to Dr. Van Leeuwen.

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6. We recommend that Dr. Welling refrain from self-prescribing medications.

I do not prescribe any psychiatric medications to myself. I only take the psychiatric medications prescribed by Dr. Van Leeuwen. Dr. Van Leeuwen has reviewed all other prescriptions for chronic medical conditions including medications for adult-onset diabetes, high blood pressure, and hyperlipidemia to ensure that there are no drug-drug interactions with his regimen of Abilify and Escitalopram.

7. The assessment team recommends that Dr. Welling practice all time in full accordance with relevant federal, state, local, organizational, and professional regulations, ethical guidelines, and best practices. If he ever is unable or unwilling to adhere to these recommendations, we recommend that he disengage himself from the practice of medicine immediately.

My goal is to continually improve my practice and serve my patients and my surgeon colleagues with attentive, accurate, and "best practices" in my anesthesia services. I continue to read articles to maintain current in my practice an in hopes that my care (when I start working again) will be consistent with my peers and colleagues. I am motivated to return to medicine and regain the reputation of a skilled, alert and attentive anesthesiologist who provides an excellent, safe anesthetic course for his patients and efficient services for my surgeon colleagues. I do not plan on retiring from medicine for another 10 years. Therefore, have committed to continuous improvement in my anesthesia delivery and services.

I have compiled some recommended "course corrections" from the experience with the Davis Hospital MEC and included many of their suggestions in my future practice. I have also reached out to my former partners at Davis Hospital and asked them for their feedback and observations regarding practice habits that I can improve. I have placed this list of my phone for frequent review and future revisions, if indicated. These "learned lessons" include the following:

DISTRACTONS. Avoid distractions from patient care in the OR. These distractions include my personal cellphone (NOT OT BE USED WHILE IN THE OR). Maintain "work focus" mode on cell phone during cases. Keep cell phone in backpack or locker-not on anesthesia machine or pyxis. Computers and anesthesia, charting programs-these can also distract from patient care. Always maintain patient-centric focus.

CHARTING. Chart all medications immediately when removed from the Pyxis. Reconcile all medications at the beginning of the case, the middle (during free moments) and at the end of the case.

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VIGILANCE. Be vigilant in the OR. When you are not attending to the patient or the patient's chart, keep your eyes on the surgeon. Try to anticipate his needs with the patient's surgical condition.
TIMELINESS. Arrive 15-30 min early in the OR. Arrive early for all appointments and surgery times.

SEDATION. Do noy sedate patients unless the patient request sedation. Notify nurses when sedation given. Remain with patient after sedation given to observe effects and depth of sedation. Do not oversedate patients.

ACCOUNTABILITY. Take responsibility for errors. Be transparent and approachable. Request feedback from surgeons, OR circulating nurse, anesthesia colleagues.

COMMUNICATE. Seek feedback from surgeons at the beginning of the case to see if there are unusual circumstances that the surgeon anticipates or specific requests from the surgeon. Seek feedback on surgical conditions after the surgery is concluded. Seek feedback on how to improve the surgeon's experience.

ORGANIZATION. Keep workplace clean and uncluttered. No medication vials or syringes out on anesthesia machine.

ANTICIPATION. Anticipate end of case and surgeon's needs during the different phases of the procedure.

BE VISIBLE. Wait in the OR for the next case-not in the lounge. Be visible and easily found. Go to the bathroom before each case. Plan ahead.

I have another two pages of notes and observations that are specific recommendations for how to handle the nuances of the hospital EMR and anesthesia record-keeping system. I will not include those since they are specific to the anesthesia EMR.

So Mr. Hibbler, I have also taken Acumen's recommendations seriously. I had to pay over $9000 for this assessment and I wanted to get as much benefit from their observations and suggestions as possible. My goal is to continuously improve my medical practice and the care which I provide patients as well as the services I provide to my surgeon colleagues.

If you have additional feedback or observations that you wish to share, please respond in kind. I have also included my personal cell phone number should you wish to speak with me directly.

I sincerely hope that I've adequately addressed your concerns regarding my Wyoming medical license. Although I have no immediate plans to practice in

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Wyoming, I love the state and would welcome the opportunity to practice again if an opening for an anesthesiologist occurs.

I look forward to our telephone conference on Thursday, April 21 at 4:30PM.

Best regards,

Eric Welling, MD MBA

XLIV. On April 21, 2022, Dr. Welling met for an Informal Interview with the Interviewers and the Board's Prosecuting Attorney.

XLV. On July 6, 2022, the Interviewers requested the Board officers order Dr. Welling to submit to an updated mental health examination.

XLVI. On July 22, 2022, the Board Officers entered *Board Officers' Order for Alcohol, Substance and Mental Health Examination of Eric Welling, M.D., Wyoming Physician License No. 10015A* (Mental Health Examination Order).

XLVII. Pursuant to the Mental Health Examination Order, Dr. Welling was ordered to complete the examination no later than Friday, September 9, 2022.

XLVIII. On July 22, 2022, Board staff e-mailed Dr. Welling a copy of the Mental Health Examination Order. Board staff also sent a copy via first class mail.

Dr. Welling's Renewal Application for his Wyoming Physician License

XLIX. On October 27, 2021, the Utah Board began investigating Dr. Welling.

L. On May 19, 2022, the Board office received Dr. Welling's application for renewal of his Wyoming Physician License.

LI. On his renewal application, Dr. Welling answer "No" when asked:

Question B: Do you at this time have any medical condition which, in any way, impairs or limits, or might impair or limit your ability to safely and skillfully practice as a physician?

Question C: Since you last renewed your license (or this is the first time renewing your license, since the date your license was originally issued) have

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you been hospitalized for, missed work because of, or been significantly impaired by a metal or emotional condition?

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Question F: Since you last renewed your license (or this is the first time renewing your license, since the date your license was originally issued) have you been under investigation or had or have any adverse charges or complaints filed against you by: any education training program or facility; medical licensing board; local, state, federal or military professional or disciplinary body; any hospital privileges or credentialing body or grievance committee; or any other medical group, including medical societies and specialty boards?

LII. On his renewal application, Dr. Welling answer "Yes" when asked:

Question E: Since you last renewed you license (or this is the first time renewing your license, since the date your license was originally issued) have you been reprimanded, demoted, disciplined, cautioned, placed on probation, been placed on or taken leave (except vacation leave), or been terminated by any employer, educational institution or training program for any reason?

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Question G: Since you last renewed you license (or this is the first time renewing your license, since the date your license was originally issued) have you been denied licensure, privileges or membership by any licensing board, hospital, medical facility, professional society, specialty board, or medical body?

LIII. On his renewal application, Dr. Welling explained that he was suspended from Davis Hospital for medication discrepancies resulting from new EMR systems. He explained that he felt the length of the suspension because it was reported. He elected to take an indefinite medical leave of absence and decided not to work at Davis Hospital again.

Dr. Gadbois' Evaluation of Dr. Welling-Unfit to Practice

LIV. Dr. Welling submitted an application for licensure in Florida.

LV. Professional Resource Network (PRN) referred Dr. Welling to Brian Gadbois, M.D., a licensed Psychiatrist, for an *Independent Medical Evaluation (IME)* (Dr. Gadbois' Evaluation).

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LVI. On September 6, 2022, Dr. Welling presented to Dr. Gadbois.

LVII. Dr. Gadbois noted that:

i. The Acumen Report made several recommendations which, upon referral today, it was unclear if they were followed.

ii. Further, upon referral to Florida PRN by the Florida Board if Medicine, Dr. Welling had not established that he followed the treatment recommendations set forth by Acumen, which is a reputable and established evaluation center specializing in physician impairment.

iii. IT is clear from available documentation that Dr. Welling's employer was relying on the Acumen report and seemingly expected Dr. Welling to follow Acumen's recommendations.

iv. Dr. Welling shared that he does not have a current treating primary care physician but has consulted with both his cardiologist, Dr. Daniel Humiston and his psychiatrist, Dr. Van Leeuwen.

v. Dr. Welling reports he is currently self-prescribing Metformin 500mg 1 tablet PO BID, Amlodipine 100mg 1 tablet daily, Rosuvastatin 10mg 1 tablet PO QHS, Famotidine (OTC) 50mg PO BID, and Niacin (OTC) 500mg 1 tablet PO QHS out of precaution due to daily history.

vi. Depression 40 years old (self-diagnosed) found self to be more irritable with wife and children, fund self not as good provider. Put self on Prozac at that time. Helped with more ability to think prior to reaction to stressful situations.

LVIII. Dr. Gadbois' Evaluation provided:

IMPRESSION:

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One focal point of Dr. Welling's case, at this time, is the extent of his follow through with Acumen's recommendations from 11/8/21. His follow through is summarized below:

1. Medication adjustments (there is ample evidence that this was accomplished and he remains under the care of Dr. Van Leeuwen, his psychiatrist-however it is unclear if Dr. Van Leeuwen was aware of the results of Dr. Welling's evaluation or recommendations from Acumen.

2. Leave of absence from work until further assessment demonstrates an improvement in his thought organization on the medication (there is evidence from his employer that he requested and was granted an extended leave of absence based on the findings of his assessment from Acumen).

3. Intensive partial hospitalization or day treatment program for professionals (there is no evidence of treatment or attempted treatment at the PHP level of care or any other level of care).

4. Treatment with a psychologist to address personality issues noted (there is no evidence of psychological treatment/psychotherapy since leaving Acumen).

5. Authorize ROI for the Acumen evaluation report to appropriate parties (unknown, we do know he release the report to us).

6. Refrain from self-prescribing medications (unknown).

7. Practice only within established legal, ethical and professional boundaries (unknown, he has not been under PHP monitoring).

The other focal point is on evidence of current impairment. Upon referral to Acumen for assessment in 2021, there was a never a concern for drug diversion from his employer. Subsequent lab testing also was in favor of no apparent substance use. Dr. Welling however did call attention to potential mental impairment/confusion/deficits that could have caused his ongoing charting problems. Upon evaluation at Acumen, a diagnosis of cyclothymia, potentially unspecific bipolar as well as clinically significant neurocognitive finds and personality issues led to a determination that Dr. Welling was not safe to immediately return to practice, but rather should engage in an appropriate treatment program.

Lack of insight is a hallmark of bipolar disorder. At this time Dr. Welling seems to have no insight that he should have followed the recommended plan. Instead he applied for a license in another State which gives the appearance of attempting to get around the recommendations. At thit time, given the context of the situation developing with Dr. Welling over the past year, it cannot be said with medical certainty that anything has changed other than he appears to be compliant with medications prescribed his psychiatrist.

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DSM V DIAGNOSIS

1. Cyclothymia with anxious features

2. Cluster B personality disorder (per Acumen assessments in 2021)

3. Compulsive behavior related to internet (rule-out process addiction)

STATEMENT OF POTENTIAL FOR IMPAIRMENT: With the diagnosis of cyclothymia, with prior related workplace impairment and inadequate follow-through with treatment recommendations, an impairment has bene established. The established impairment creates an elevated risk to the public.

SAFETY TO PRACTICE STATEMENT: Eric Welling is not able to practice his profession with reasonable skill and safety at the time of the evaluation. Due to concerns noted in Dr. Welling's neurocognitive testing a year ago and recommendation from Acumen that "Dr. Welling should take a leave of absence from work until further assessment demonstrates an improvement in his thought organization on the medication," repeat neurocognitive testing should be considered as part of this process.

SUMMARY: The recommended course of action for Eric Welling is to enter a PRN-approved longitudinal professionals program, specializing in the treatment of mental health disorders in physicians.

TREATMENT RECCOMENDATIONS:

1. Eric Welling should enter and complete a PRN-approved longitudinal professionals program.

2. Comprehensive neurocognitive testing is recommended, as part of the treatment planning and return to work process.

3. Eric Welling should engage in individual outpatient psychotherapy for recovery support and to address personality issues identified in the Acumen assessment.

4. All controlled medications should be discontinued and/or avoided due to inherent abuse/dependence potential. Eric Welling should refrain from self-prescribing medications.

5. Eric Welling is appropriate for PRN monitoring, under mental health contract.

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LIX. Dr. Welling did not comply with the Mental Health Examination Order.

LX. On November 4, 2022, the Board entered an *Order of Summary Suspension of Eric C. Welling, M.D., Wyoming Physician License No. 10015A, Pending a Contested Case Hearing*, fully incorporated herein.

LXI. On November 20, 2022, the Board office received D. Welling's e-mailed response again detailing each of the Acumen recommendations and how he felt he had strictly complied with the recommendations:

> I feel it is important to answer some of the Board's questions about my suspension from Davis Hospital and the subsequent investigation, and subsequent Summary Suspension of my Wyoming Medical license by the Wyoming Board of Medicine. I would like to formally appeal the Summary Suspension by the Wyoming Board of Medicine.
>
> Shortly after the suspension was reported in the NPDB, I was informed by the Wyoming Board of Medicine of their own investigation. In my opinion, it would have been reasonable for them to contact the Utah Division of Professional Licensing to inquire about their investigation findings and rulings. I believe it was outside of the purview of the Wyoming Board of Medicine to reinvestigate the same allegations, examinations, and rulings but then arrive at a different conclusion and require another set of examinations and evaluations by Acumen Assessments. I believe it is unreasonable for the Wyoming Board of Medicine to mandate where I go fore treatment. The Wyoming Board did not make the diagnosis. They should not prescribe the therapy or to whom I go for therapy. I believe that a repeat investigation months after the events at Davis Hospital which resulted in the suspension constitutes a sort of "double jeopardy." I also believe that they are interfering with my treatment by ordering me back to Acumen Assessments instead of allowing me to pursue treatment from other therapists (acceptable to Dr. Van Leeuwen and Dr. Stacy).
>
> The suspension from Davis Hospital and my subsequent indefinite medical leave of absence was thoroughly investigated by the Utah Department of Professional Licensing and interviewed me, Brittany Dietrich (Davis Hospital Medical Staff Coordinator), Dr. Stacy of Acumen Assessments, and Dr. Van Leeuwen, psychiatrist, Tanner Clinic. The matter was investigated and adjudicated by Shari Scheeler, investigator and Elizabeth Sorenson, counsel, for the Utah Board of Medicine. The ruling from the Board of Medicine was that since the suspension did not involve any type of diversion, but involved a treatable mental health condition, that as long as I complied with psychiatric treatment and followed the recommendations of my psychiatrist and that I did

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not repeat the behaviors which resulted in the investigation at Davis Hospital, that my medical license with the State of Utah would continue to be valid and in good standing.

These are the original conclusions and recommendations from Acumen Assessments:

Recommendations of Acumen Assessments:

1. Dr. Welling may benefit from a trial of Lamictal as primary mood stabilizer. It will take time to titrate him up to a therapeutic dose, so an augmentation strategy with a second-generation antipsychotic medication may be helpful. It is not clear if Wellbutrin and Buspar are doing much. Dr Whipple suggests discontinuing these medications to simplify the regimen. At this point, we would address his difficulties as if he is developing a late-life bipolar condition.

2. Dr. Welling should take a leave of absence from work until further assessment demonstrates an improvement in his thought organization on the medication.

3. Dr. Welling would benefit from an intensive partial hospital or day treatment program for professionals in crisis.

4. We recommend that Dr. Welling acquire a local psychologist to address the personality issues noted in this report.

5. We recommend that Dr. Welling sign releases of information to facility sending this report to his providers.

6. We recommend that Dr. Welling refrain from self-prescribing medications.

7. The assessment team recommends that Dr. Welling practice all time in full accordance with relevant federal, state, local, organizational, and professional regulations, ethical guidelines, and best practices. If he ever is unable or unwilling to adhere to these recommendations, we recommend that he disengage himself from the practice of medicine immediately.

I have followed, or am in the process of following, all of these recommendations including the following:

Acumen Assessment agreed with my choice of Benjamin Van Leeuwen, DO, Psychiatrist at the Tanner Clinic, for follow-up and medication management. Dr. Van Leeuwen is the only physician managing my psychiatric medications. After examining the Acumen Assessment report, discussing my case with Dr. Stacy (AA), Dr. Van Leeuwen recommended Abilify 4mg as a mood stabilizer and the antidepressant, Escitalopram

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30mg. I have seen Dr. Leeuwen for the past year and have been compliant with the medication and with my appointments.

In addition, I met with Dr. Brian Gadbois, Psychiatrist, Board Prep Recovery Center, Tampa, FL. Part of this evaluation was a forensic toxicology screen including blood, urine and hair analysis. All of these results demonstrated no exposure to ANY illegal or controlled substances.

After I informed the Wyoming Board of Medicine of my decision to consult with Dr. Gadbois, the Wyoming Board of Medicine stated that they would not accept my consultation with Dr. Gadbois as an acceptable physician to monitor my progress and stated that they required that I return to Acumen Assessments for follow-up and another evaluation. Acumen Assessments did not require me to return to AA for follow-up or treatments. They recommended continuing sessions with Dr. Van Leeuwen and a local psychologist for therapy. It is unreasonable that the Wyoming Board of Medicine go against the counsel of Scott Stacy, Director of Acumen Assessments by requiring me to return to Acumen for ongoing therapy.

I took a nearly 7 month leave of absence from Anesthesia from December 12, 2021 until June 15, 2022. During this leave of absence, I continued to meet with Dr. Van Leeuwen and continued to take my medications as prescribed by Dr. Van Leeuwen. I rested, healed, and looked for future employment opportunities.

I found and met with a mental health counselor in Salt Lake City, Jeffrey Riekhof. I had a preliminary meeting with him after being on a waiting list to see him for two months. I share with him the Acumen Assessment report. Upon further examination, I learned that Dr. Reikhof was not a Psychologist, but rather a mental health counselor (Sociologist). Wanting to comply with the Acumen Assessment's recommendations, I asked Dr. Stacy at Acumen Assessments to share with me a list of approved Psychologists that were experienced with Medical Staff issues. He provided me with several resources. I have struggled to find a psychologist with whom to meet during COVID, but have since found several options.

I have refrained from writing any psychiatric medications for myself. My only prescriber for these medications is Dr. Benjamin Van Leeuwen.

Allegations of Alcohol and/or drug use. During my focused review at Davis Hospital, I was tested for drugs on several occasions. All of these tests were negative. Despite Davis Hospital having their suspicions satisfied that there was no alcohol use or drug diversion, Acumen Assessments again tested me for drugs (all testing was negative) and administered a lie detector test (which was also negative when questioned about drug diversion). During an evaluation in September 2022, I was again given a toxicology screen including blood, urine and hair sample

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analysis. None of these samples were positive for drugs. The Davis Hospital suspension did not involve drugs or alcohol. It was the result of a mental disorder which made it difficult for me to do my anesthesia charting on confusing and incompatible EMRs.

What is more important is the following:

> That I am compliant with the original recommendations made by Acumen Assessments-which I am.
>
> That I am compliant with the medications prescribed by Dr. Van Leeuwen-which I am. My next appointment with Dr. Van Leeuwen (first available) is 1/13/23.
>
> That I have taken some time off work to heal and t consider the counsel and treatment that I have been given-I took 6 months off from work from December 2021 to June 2022.
>
> I have had a difficult time in finding a psychologist therapist that would be able to work with Dr. Van Leeuwen. The psychologist in Dr. Van Leeuwen's office left for another position and his position with Tanner Clinic remains vacant. I continue to look for another therapist and have found an option that may satisfy that stipulation particularly now that I have moved to Las Vegas, NV to work.
>
> Most important that the prescribed medications and therapy resulted in improvement in my mood and ability to practice medicine. I have successfully worked at a Surgery Center (Northern Michigan Surgery Suites, Boyne City, MI) and a hospital system (Sunrise Hospital, Flamingo Surgery Center, Las Vegas, NV). I've noticed an improvement in my attention and work quality. I have worked with several new computer charting systems and have experienced no errors in anesthesia charting or medication reconciliation. I have had several compliments from the surgery centers and how they enjoy working with me as well as surgeons who have been complimentary of my work.
>
> I do not prescribe my own medications, with the exception of blood pressure and anti-cholesterol medications.
>
> Most importantly, I feel that my evaluation at Acumen brought to my attention some areas where I needed to improve my personality and in my work habits. I have tried to be objective and teachable about these areas of needed improvement and have tried to address each one individually. I feel that I have been successful in implanting the needed improvements demonstrated by my successful work in a hospital and several surgery centers It is my

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professional goal to continue to be compliant with the recommendations of Acumen Assessments and Dr. Van Leeuwen.

PRC's Assessment of Dr. Welling-Unfit to Practice

LXII. On January 6 through 9, 2023, Dr. Welling presented to PRC for the comprehensive multidisciplinary evaluation requested by the Board.

LXIII. PRC issued an *Assessment Discharge Summary* (PRC Report).

LXIV. In its report, PRC noted that:

i. Throughout the evaluation Dr. Welling demonstrated difficulty maintaining a cohesive and sequential narrative of his work history and elements of his behavior that others might consider to be problematic.

ii. He provided PRC with a resume, which was not consistent with information he provided PRC clinicians during clinical interviews including one that focuses on occupational history.

iii. He provided confusing and sometimes contradictory information.

iv. During clinical interviews he tended to rationalize, justify, deny, and was defensive.

v. HE also demonstrated a tendency to externalize blame.

LXV. When he was asked about reasons for not following through with the Acumen recommendation that he participate in a professionals program he initially said that he and his psychiatrist viewed this as not necessary. He later reported to PRC clinical staff that he did not participate in the recommended programing because he could not afford such a program. In correspondence with the Board, he asserted that Acumen was self-referring, implying he had no actual need for such services.

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LXVI. Another example of problematic behavior is that despite having discussions with clinical staff at Acumen and the Board around prescribing, he continues to call in prescriptions for family members. Per his report, Dr. Welling will write/call in his own medications after consulting with his cardiologist who he views as his primary care provider.

LXVII. Additional areas of concerns are related to his communication and engagement with the Board. PRC noted that Dr. Welling has not been consistently timely in responding to them, has not been honest in some of the information provided about obtaining an evaluation, and has not been compliant with the Acumen aftercare recommendations.

LXVIII. Dr. Welling had started individual psychotherapy (although he only attended two sessions in 2022, six months apart and was not in compliance with the therapist's expectation of frequency of treatment visits) and he had made substantial changes in his work environment.

LXIX. PRC noted that given his history of self-prescribing it is questionable whether he has a primary care physician.

LXX. The PRC report noted:

MENTAL STATUS EXAMINATION:

According to PRC's consulting psychiatrist: His speech was quite rapid. He was inconsistent in his answers to questions and often provided answers in a defensive and rationalizing manner. He would make comments that were unclear and required clarification. Upon further discussion there seemed to be externalizing of blame and variable assumption of responsibility. He was unclear about specifics, particularly Acumen's recommendations. He also made comments he could not afford services Acumen was offering. Dr. Welling apparently felt that seeing a community psychiatrist and getting medication was sufficient to meet Acumen's recommendation. Additionally, he thought he was getting therapy from his psychiatrist, which his psychiatrist later explained was not the case, rather he would need to get therapy from a

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provider other than his psychiatrist. Per his report, he indicated his psychiatrist has been reporting to the Wyoming Board of Medicine.

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Medical Concerns: Information about his medical history was not consistent as he provided different information on his client information form and clinical interviews. Dr. Welling reported during clinical interviews that he has not been diagnosed with specific medical conditions; however, he is treating himself for high blood pressure and cholesterol. He stated he consults with his cardiologist and then self-prescribes his medications. He is currently self-prescribing Amlodipine 10mg, Metformin 500mg BID, Rosuvastatin 10mg, Niacin 500mg, COQ enzyme 200 mg, Famotidine 100 mg b.i.d, and Acyclovir 400mg. He stated he self-prescribed Acyclovir as he read research that indicated there may be association between taking an antiviral medication and reducing the risk of developing Alzheimer's dementia.

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Psychiatric Concerns: Dr. Welling reported he historically has had concerns related to depression. He reported he started to self-prescribe psychiatric medication such as Prozac and then shifting to Wellbutrin and Lexapro. Reportedly, he started seeing a psychiatrist in September of 2021. He reported his psychiatrist prescribes and writes his (Dr. Welling's) psychiatric medication prescriptions. Per Dr. Welling's report, he is prescribed Lexapro 30 mg and Abilify 5 mg. According to records provided by the psychiatrist, Abilify is prescribed at 2 mg. It is PRC's consulting psychiatrist's clinical opinion that these dosages are not at an optimal level.

In addition to seeing the community psychiatrist he (Dr. Welling) meets with approximately once every three months, Dr. Welling reported he started seeing an individual psychotherapist. Per Dr. Welling's report, the first session occurred in June of 2022. Seeing a therapist was one of the recommendations made by Acumen. Dr. Welling stated he again met with his therapist in December of 2022.Both progress notes from these sessions indicate the therapist's intention was to see Dr. Welling on a more regular basis.

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Overall results of the current evaluation were consistent with neuropsychological findings obtained during the Acumen evaluation.

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Dr. Welling's difficulties with compliance mentioned above, including his continuing to work when recommended not to and his lack of tending to his own health and wellbeing are seen as major deficiencies in the ABMS core competency area of Professionalism. During the evaluation at PRC, the PRC

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clinical team observed other deficiencies in Professionalism as he demonstrated behaviors similar to behaviors observed at Acumen. During clinical interviews at PRC tended to rationalize, justify, deny, or was defensive. He had difficulty accepting responsibility for his behaviors, would tend to externalize blame and demonstrated poor insight into the causes and potential effects of the problematic behavior. In response to the PRC clinical team's inquiries about why he did not follow through with Acumen's recommendation to participate in a professionals program he initially stated it was because his psychiatrist reviewed his records and decided it was unnecessary, although later in the evaluation process he reported that his primary motivation for not completing the professionals program was because of cost.

Another area of concern indicative of deficiencies in the ABMS core competency area of Professionalism is that he engages in poor self-care and has a lack of recognition of how his physical and mental wellbeing might impact patient care. For example, he does not appear to have appropriate downtime pre or post call. This in turn may be at least partially a symptom of his underlying Bipolar Spectrum Disorder as he may be overly goal directed.

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Dr. Welling's struggles to recognize and manage both internal and external triggers. He has poor self-reflective awareness, demonstrates poor judgment, and fails to anticipate the consequences of his actions. These factors likely contribute to him making poor decisions and engaging in behaviors that are not in his best interest in the long term (for example, not responding to the Wyoming Board of Medicine communications/orders timely).

His willingness to seek out treatment or engage in the change process appears to be primarily externally motivated. He had been prescribing himself psychiatric medications until recently. When eventually sought psychiatric assistance, he was somewhat reluctant in following the psychiatrist's and Acumen's recommendations that he establish a relationship with a therapist, engage in individual psychotherapy, and he delayed following through with this recommendation by a least six months. When he finally did engage in psychotherapy, he did so secondary to the Wyoming Board of Medicine's concern about his lack of compliance with the Acumen recommendations. He has only seen his individual psychotherapist twice in 2022 and there were approximately six months between sessions. Dr. Welling's reluctance to comply, his tendency to either withhold, minimize, and/or distort information are all concerning and would suggest that he is in the pre-contemplative stage of change.

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It is likely that Dr. Welling will continue to struggle with elements of his behavior without more extensive assistance. While he reported he has sought out psychiatric and psychological treatment, collateral data indicate his

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participation in therapy has been minimal. As he is a poor historian it might be difficult for his psychiatrist to be fully aware of all the issues that Dr. Welling has been experiencing.

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FITNESS TO PRACTICE AND RECCOMENDATION:

Results of the current evaluation are similar to findings obtained during his Acumen evaluation in October of 2021. Data indicate he has demonstrated poor judgment in both his personal and professional life. Information obtained through collateral sources indicates that Dr. Welling has a long history of difficulties at the workplace.

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Recommendations:

1. Participating in treatment in a treatment program at the partial hospital or day treatment level of care prior to a return to the clinical practice of medicine. Important components of his treatment would include psychiatric oversight/medication management as well as participation in individual and group therapy sessions. The treatment program should have familiarity in working with medical professionals with similar issues (mood disorders, professionalism concerns, personality characteristics, ect.) We would recommend hat Dr. Welling discuss appropriate facilities with the Board prior to initiating treatment.

2. It will be important that following the completion of the treatment mentioned above he has a comprehensive aftercare plan including regular psychiatric follow up and regular psychological follow-up. It will be important that he fully complies with all aftercare recommendations.

3. Dr. Welling needs to establish a relationship with a primary care provider who should be responsible for writing all Dr. Welling's prescriptions.

4. Dr. Welling need refrain from self-prescribing medications and refrain from writing prescriptions for family members.

5. The team would be happy to provide this report to his treaters with appropriate releases in place.

6. He needs to comply with all recommendations and requirements as set forth by the Wyoming Board of Medicine.

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Dr. Welling Presents to Dr. Bone-Psychologist

LXXI. Jonathon Bone, Psy. D., is a License Psychologist.

LXXII.On February 16, 2023, Dr. Welling presented to Dr. Bone. They discussed "[e]xercise; avoiding burnout; continue to make it back to SLC every other month; ordered Rapt-present focus book; walking [.]"

LXXIII. On February 23, 2023, Dr. Welling presented to Dr. Bone. They discussed "Burn out; board issues; need for a new psychiatrist."

LXXIV. On March 2, 2023, Dr. Welling presented to Dr. Bone. They discussed "Corrective behaviors; maintaining stable mood; upcoming hearing on March 13 and need to prepare documentation; direction; need for self-care [.]"

LXXV. On March 9, 2023, Dr. Welling presented to Dr. Bone. They discussed "Pre-hearing document; treatment needs; addressing burnout; self-care."

Mr. Chase's Assessment of Dr. Welling-Conditional Fitness to Practice

LXXVI. In March 2023, Dr. Welling presented to Mark Chase for a Psychological Assessment. The Nevada State Board of Medical Examiners requested a psychological and fitness-to-practice assessment of Dr. Welling.

LXXVII. Mark Chase is the Clinical Program Director for PRN. He holds M.A. and M.B.A. He is a Doctoral Candidate in Neuropsychology.

LXXVIII. Mr. Chase noted:

> **DASS-21 Assessment:** The DASS 21 is a self-report questionnaire designed to measure the severity of a range of symptoms common to depression, anxiety, and stress. The DASS-21 assessment completed by Eric for general anxiety, stress, and depression indicated very little symptomatology for anxiety, depression, or stress.
>
> **Novaco Anger Assessment:** The disposition of anger has been considered cognitive, arousal, and behavioral. It describes someone's ability to regulate anger when provoked an anger is aroused. Eric scored well on the assessment, making him more peaceful than average.

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Adult ADHD (ASRS-V1.1) Assessment: Eric did not meet the adult attention hyperactivity disorder criteria.

Behavioral Observations: Throughout the evaluations, Eric demonstrated a cohesive and sequential narrative of his educational, work, and personal life history. During the discussion about the event that had prompted this assessment, Eric never rationalized or justified the facts but was straightforward in his explanation. Eric expressed regard about not following through with ensuring the Wyoming Board had the requested information and felt the whole situation was a wake-up call to him about work-life balance. No narcissistic behaviors or attitudes were observed during the evaluation.

LXXIX. Mr. Chase provided the following recommendations:

Recommendations:

1. Eric will participate in the PRN Professionals Aftercare/Monitoring Program for twelve months. Weekly meetings with other professionals will allow Eric to develop a robust support system with other physicians who share similar challenges in the workplace. Furthermore, monthly one-on-one meetings will give Eric an additional resource to process personal and professional stressors.

2. Eric will identify and continue to work with his psychiatrist on medical management, agreeing to meet at least every six months. Eric has identified Dr. Benjamin Van Leeuwen as his treating psychiatrist; however, that may change soon.

3. Eric will identify and continue to work with his psychological on anxiety and behavioral issues to refine his professionalism, particularly when it comes to dual relationships. Eric needs to meet at least monthly with his psychologist. Eric has identified Jonathan Bone as his treating psychologist.

4. Eric will identify a primary care provider and have that provider be responsible for writing all of Dr. Welling's non-psychiatric medications. Eric will refrain from self-prescribing medications. At present, Eric has identified Craig Harmon as his treating primary care provider.

5. Eric will work with PRN in developing and refining an appropriate self-care plan to improve his mental and physical well-being. Dr. Welling has given self-care greater importance in his life and has already seen positive results. IN the past six months at Sunrise Hospital, he has reconciled all medications exactly, has no charting

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errors, and his interpersonal communications and relationships with co-workers have ben pleasant and professional.

Fitness to Practice: Psychological Dr. Eric Welling is safe to practice and can exercise hospital privileges without restrictions as long as he complies with recommendations presented in this report.

Dr. Welling Presents to Dr. Harmon-Primary Care Provider

LXXX. Craig Harmon, M.D., is a licensed physician in family internal medicine.

LXXXI. On April 4, 2023, Dr. Welling presented to Dr. Harmon.

Dr. Welling Presents to Dr. Caten-Psychiatrist

LXXXII. Eric Caten, M.D., is a Licensed Psychiatrist.

LXXXIII. On May 25, 2023, Dr. Welling presented to Dr. Caten. Dr. Caten recommended that Dr. Welling see a psychologist in Las Vegas because Dr. Bone is located in Utah and Dr. Welling is unable to see him as frequently as he should (on a biweekly basis). He recommended individual, family or group therapy.

LXXXIV. When asked about his current medications, Dr. Welling responded, "I put myself on those medication but now Craig Harmon gives them to me."

Dr. Welling's Non-Compliance with Treatment Recommendations

LXXXV. On October 18, 2021, Dr. Welling presented to Dr. Van Leeuwen. He reported that "he has stopped the Wellbutrin as well as the buspirone as he does not think that it was doing anything for him. Has also reduced the dose of the Lexapro as well."

LXXXVI. On November 16, 2021, Dr. Welling presented to Dr. Van Leeuwen. He reported that he is "planning on resigning from DH today, and will be wor5king out of state for some time."

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LXXXVII. On February 15, 2022, Dr. Welling presented to Dr. Van Leeuwen. He reported that he "[h]as not been able to find a job for 5 months" which "has really been frustrating for him."

LXXXVIII. On May 9, 2022, Dr. Welling presented to Dr. Van Leeuwen. Dr Van Leeuwen noted:

> Has picked up rx for Abilify, but has not picked up rx for Lexapro since January 2022. Do not know why he is being deceitful about this. For now, will continue with meds at current doses. Strongly encouraged pt to find a therapist as was recommended by the Acumen evaluation. Made clear to pt that our intermittent visits do not nor have they ever constituted psychotherapy.

LXXXIX. On August 1, 2022, Dr. Welling became employed at U.S. Anesthesia Partners in Las Vegas, Nevada, full-time.

XC. On December 2, 2022, Dr. Welling presented to Dr. Van Leeuwen. He reported that recently started seeing a therapist, Jeffrey Riekhof, Licensed Clinical Social Worker (LCSW). He is also in the process of getting in with a forensic psychologist. He is working in Las Vegas full time.

XCI. On February 7, 2023, the Board Prosecutor e-mailed Dr. Welling, approving Jonathon Bone, Psy. D., Licensed Psychologist, as a psychological provider. The Board Prosecutor did not approve Mr. Chase to manage Dr. Welling's medication because Mr. Chase does not hold a medical license (M.D. or D.O.), rather he is a doctoral level "candidate." Additionally, Dr. Welling was advised that "meeting once per week does not satisfy a 'day treatment level' as required."

XCII. On March 9, 2023, Dr. Welling e-mailed Dr. Scott Stacy from Acumen regarding the recommendation for a partial hospitalization treatment program. Dr. Stacy state, "Since you have been working since last June, I assume you

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underwent another fitness-for-duty evaluation after medication changes/treatment and psychotherapy (Recommendations #1 and #4).

B. Testimonial Evidence

Witness Testimony-Dr. Fielden (Deposition)

XCIII. Scott Fielden, M.D., submitted to a deposition on May 17, 2023.

XCIV. Dr. Fielden testified he has been employed with U.S. Anesthesia Partners since 2016. He is the chairman of the clinical governing board.

XCV. When asked why Dr. Welling does not practice in Utah despite his family live there, Dr. Fielden stated, "I think because he does not have a license there."

XCVI. Dr. Fielden was not aware of the reason for the Davis Hospital suspension of Dr. Welling's clinical privileges. He was aware that Dr. Welling lost his licensure in multiple states, but did not know which jurisdictions or why. He was not aware of the recommendations following the three different psychological evaluations. He is not aware of whether Dr. Welling complied with those recommendations. He does not have any knowledge of Dr. Welling's present psychological or mental health status.

Witness Testimony-Dr. Magnus (Deposition)

XCVII. Dustin Magnus, M.D., submitted to a deposition on May 17, 2023.

XCVIII. Dr. Magnus testified he is employed with U.S. Anesthesia Partners and he is the Chief of Anesthesia at Sunrise Hospital in Nevada.

XCIX. Dr. Magnus was not aware of why Dr. Welling's physician license was suspended and lost in several states. He did not ask for copies of the different jurisdiction's' disciplinary actions. He has not reviewed the Acumen Report. He only reviewed the evaluation that said Dr. Welling was fit for practice.

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Witness Testimony-Dr. Welling

C. Dr. Welling described his professional interactions with Dr. Van Leeuwen as Dr. Van Leeuwen "acquiesce[ing] to my suggestions and thought it was a good idea that we wait until we got a diagnosis before he issued treatment." At the time of the Acumen evaluation, he was on two different antidepressants. It was suggested that he speak to his psychiatrist regarding a change in his medications. He claimed to be meeting with DR. Van Leeuwen "every week or every two weeks [.]" He did not believe Buspar was helping, so he told Dr. Van Leeuwen that he preferred to stay on the medications recommended by Acumen.

CI. He worked with Doctors Medical Center of Modesto concurrently with his employment at Davis Hospital. He continued to practice until December 2021. He started working locums in June 2022. He testified that during that six month period he was not employed, he was actively looking for work.

CII. Dr. Welling saw Jeffrey Riekhof, LCSW, starting in June 2022. He saw Mr. Riekhof three (3) times over the next seven (7) months.

CIII. He admitted that he delayed in having the neuropsychological assessment requested by the Board until January 2023 because he did not know if they would find differently from Acumen.

CIV. He testified that when Nevada became aware of the proceedings in Wyoming, the Nevada Board of Medicine wanted to follow up with their own evaluation, so he complied.

CV. He believes that letters of support are the ultimate test of fitness to practice medicine.

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CVI. Dr. Welling claims that if there would have been a day treatment program or intensive partial hospital program in Utah, he would have attended. Later, he claims that Acumen and PRC offered programs, but he thought they were self-serving.

CVII. On December 5, 2021, Dr. Welling signed a stipulation with the state of Utah voluntarily agreeing not to practice in Utah. His license was later reinstated.

CVIII. Dr. Welling testified that he did not give Mr. Chase Dr. Gadbois' Evaluation because he didn't think it was relevant. Likewise, he did not provide Dr. Caten medical records from Dr. Van Leeuwen, Dr. Gadbois' Evaluation, or Mr. Chase's assessment because it was not relevant.

CIX. He withdrew his application in Florida because "they kept throwing out things that it wanted me to do that I thought were redundant." After being hired at Sunrise Hospital, he did not need the locums assignments.

CX. Dr. Welling admitted he answered Question F on his application incorrectly.

Witness Testimony-Connie Schepp, Board Investigator

CXI. Connie Schepp is the Board's Investigator.

CXII. She rebutted Dr. Welling's statements that he contacted her at various stages of the investigation.

4. On December 14, 2023, upon applying the principles and rules of law to the findings of facts, the Wyoming Medical Board lifted the summary suspension of Respondent's license and immediately suspended his medical license until at time which he shows compliance with the following recommendations:

I. Enter an intensive partial hospital or day treatment program for professionals in crisis (important components of his treatment would include psychiatric

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oversight/medication management as well as participation in individual and group therapy sessions. The treatment program should have familiarity in working with medical professionals with similar issues (mood disorders, professionalism concerns, personality characteristics, ect.)) OR PRN-approved longitudinal professionals program, specializing in the treatment of mental health disorders in physicians;

II. Demonstrate an established psychiatric relationship with a Board-approved psychiatrist medical management (meeting at least every six months);

III. Demonstrate an established psychotherapeutic relationship with a Board-approved local psychologist to work on anxiety, behavioral, and personality issues to refine his professionalism (meeting monthly);

IV. Demonstrate an established primary care relationship with a Board-approved physician (meeting as necessary for appropriate prescribing non-psychiatric medications);

V. Verification that Dr. Welling has refrained from self-prescribing medications; and

VI. Verification that Dr. Welling has refrained from prescribing medications to friends and family members.

II. GROUNDS FOR DISCIPLINE

The facts stipulated to in the Stipulations of Fact are sufficient to establish that grounds for discipline of Respondent's medical license exist. Specifically, Respondent has violated the following statutes or rules which are part of the Tennessee Medical Practice Act, (TENN. CODE ANN. § 63-6-101, *et seq.*) for which disciplinary action before and by the Board is authorized:

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1. The facts stipulated in paragraphs two (2) and three (3) *supra*, constitute a violation of TENN. CODE ANN. § 63-6-214 (b) (20):

> Disciplinary action against the licensee to practice medicine by another state or territory of the United States for any acts or omissions that would constitute grounds for discipline of a licensee licensed in this state. A certified copy of the initial or final order or other equivalent document memorializing the disciplinary action from the disciplining state or territory shall constitute prima facie evidence of a violation of this section and shall be sufficient grounds upon which to deny, restrict or condition licensure or renewal and/or discipline a licensee licensed in this state;

III. POLICY STATEMENT

The Tennessee Board of Medical Examiners takes this action in order to protect the health, safety, and welfare of the people of the State of Tennessee and ensure that the public confidence in the integrity of the medical profession is preserved.

IV. ORDER

NOW THEREFORE, Respondent, for the purpose of avoiding further administrative action with respect to this cause, agrees to the following:

2. The suspension Tennessee medical license of Eric Welling, M.D., license number 65466 lifted and is hereby **REVOKED** effective the date of entry of this Order.
3. Respondent must pay, pursuant to TENN. CODE ANN. §§ 63-6-214 (k) and 63-1-144(a)(4), the actual and reasonable costs of prosecuting this case to the extent allowed by law, including all costs assessed against the Board by the Division's Bureau of Investigations in connection with the prosecution of this matter. These costs will be established by an Assessment of

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Costs prepared and filed by counsel for the Department. Said costs shall not exceed three thousand dollars ($3,000.00).

4. Any and all costs shall be paid in full within thirty (30) days after the issuance of the Assessment of Costs unless Respondent makes arrangements for an extended payment plan for the assessed costs through the Disciplinary Coordinator of the Division of Health-Related Boards. Payment shall be made by certified check, cashier's check, or money order, payable to the State of Tennessee, Department of Health. Any and all payments shall be forwarded to the **Disciplinary Coordinator, The Division of Health-Related Boards, Tennessee Department of Health, 665 Mainstream Drive, 2nd floor, Nashville, Tennessee 37243**. A notation shall be placed on said money order or such check that it is payable for the costs of **Eric Welling, M.D., COMPLAINT NO. 17.18-231717A.**

5. Respondent understands that this is a formal disciplinary action and will be reported to the National Practitioner Data Bank (N.P.D.B.) and/or similar agency.

This **AGREED ORDER** was approved by a majority of a quorum of the Tennessee Board of Medical Examiners at a public meeting of the Board and signed this 5th day of March, 2024

Chairperson
Tennessee Board of Medical Examiners

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APPROVED FOR ENTRY:

Eric Welling, M.D.
Respondent

2/23/24
DATE

Jessica L. Turner, BPR# 032409
Senior Associate General Counsel
Office of General Counsel
Tennessee Department of Health
665 Mainstream Drive, 2nd Floor
Nashville, Tennessee 37243
(615) 289-5379

3/5/24
DATE

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CERTIFICATE OF SERVICE

The undersigned hereby certifies that a true and correct copy of this document has been served upon Respondent:

Eric Welling
6599 S. Juliet Way
Salt Lake City, UT 84121

by delivering same in the United States Mail, Certified Number *9589 0710 5270 0676 1490 48*, return receipt requested, and United States First Class Postage Pre-Paid Mail, with sufficient postage thereon to reach its destination and via email at: welling.eric@gmail.com.

Jessica L. Turner
Jessica L. Turner
Senior Associate General Counsel

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